



SURGICAL PATHOLOGY REPORT

Patient Name: [REDACTED]
Med. Rec. #: [REDACTED] Visit #: [REDACTED]
DOB: [REDACTED] Sex: Male
Soc. Sec. #: [REDACTED] Location: [REDACTED]
Physician(s): [REDACTED]

Accession #: [REDACTED]
Service Date: [REDACTED]
Received: [REDACTED]
Client: [REDACTED]

ICD-O-3

Carcinoma, urothelial NOS 8120/3
Site: Bladder wall NOS C67.9
4/23/14

FINAL PATHOLOGIC DIAGNOSIS

- A. Right ureter, biopsy: Benign ureter segment, no dysplasia, no carcinoma.
- B. Left ureter, biopsy: Benign ureter segment, no dysplasia, no carcinoma.
- C. Urinary bladder, prostate gland and bilateral seminal vesicles, cystoprostatectomy:
- Bladder:
5% squamous, per TSS.
1. Invasive high-grade urothelial carcinoma with squamous differentiation extensively involving muscularis propria (Stage T2), excised; see comment.
2. No tumor identified in one lymph node (0/1).
 - Prostate:
1. Invasive urothelial carcinoma focally within urethral subepithelial connective tissue (Stage T1); see comment.
2. Urothelial carcinoma in situ involving peri-urethral ducts; see comment.
 - Seminal vesicles: No tumor identified.
- D. Left pelvic lymph nodes, dissection: No tumor identified in sixteen lymph nodes (0/16).
- E. Right external iliac lymph nodes, dissection: Metastatic carcinoma present in three of four lymph nodes (3/4).
- F. Right obturator lymph nodes, dissection: No tumor identified in eight lymph nodes (0/8).
- G. Right internal iliac lymph node, biopsy: No tumor identified in one lymph node (0/1).

H. Left ureter, biopsy: Benign ureter segment, no dysplasia, no carcinoma.

I. Right ureter, biopsy: Benign ureter segment, no dysplasia, no carcinoma.

COMMENT:

Bladder Tumor Synoptic Comment

- **Tumor type:** Urothelial carcinoma with squamous differentiation.
- **Tumor grade:** High.
- **Tumor size:** 3.5 x 2.4 x 2.4 cm.
- **Extent of tumor in bladder:** Invasion into outer half of muscularis propria.
- **Lymphatic/vascular invasion:** None. Immunostains for the vascular marker CD31 and the lymphatic marker D2-40 were performed on block C7 and they fail to demonstrate definitive vascular invasion.
- **Epithelial abnormalities in bladder:** Carcinoma in situ.
- **Extension of tumor into adjacent organs:** None.
- **Margins:**
 - **Urethral margin:** Negative, no tumor is present on enface frozen section margin (C1).
 - **Right ureter margin:** Negative.
 - **Left ureter margin:** Negative.
 - **Perivesical margin:** Negative (tumor is 2 mm from margin, slide C7).
- **Lymph node status:** Positive.
 - total number of positive nodes: 3 (all positive lymph nodes from right external iliac).
 - total number of nodes examined: 30.
 - Size of largest metastasis in nodes: 11 mm.
 - Extranodal extension: None.
- **Other pathologic findings in bladder:** None.
- **AJCC/UICC stage:** pT2bN2.

Tumor Synoptic, Urethra

The prostatic urethra contains extensive carcinoma in situ (CIS), there are several foci of detached urothelial carcinoma within the subepithelial connective tissue with paradoxical differentiation, single cells, and jagged borders (best seen on slide C17, level 1) consistent with superficial invasion into the subepithelial connective tissue of the prostatic urethra. No tumor is seen invading from the bladder wall into the prostate. No tumor is identified at the urethral margin.

- **Tumor type:** Invasive urothelial carcinoma.
- **Tumor grade:** High grade.
- **Location of tumor:**
 - Prostatic urethra: Slides C3 and C5 (CIS only).
 - Prostate right base: C17.
 - Prostate left mid: C20 (CIS only).
 - Prostate left base; C21 (CIS only).
- **Margins:** Negative, no tumor is present on enface frozen section margin (C1).
- **Lymphatic/vascular invasion:** None.
- **Lymph node status:** Negative; total number of nodes examined: 30 (Assumes lymph node metastasis from urothelial carcinoma originating in urinary bladder).
- **Perineural invasion:** None.
- **AJCC stage:** pT1N0.

Specimen(s) Received

A:Right ureter, biopsy (FS)

B:Left ureter, biopsy (FS)

[REDACTED]

C:Bladder and prostate (FS)
D:Lymph node, left pelvic
E:Lymph node, right external iliac
F:Lymph node, right obturator
G:Lymph node, right internal iliac
H:Final left ureter
I:Final right ureter

Intraoperative Diagnosis

FS1 (A) Right ureter, biopsy: Benign ureter segment, no dysplasia, no carcinoma. [REDACTED]

FS2 (B) Left ureter, biopsy: Focal epithelial atypia, no high-grade dysplasia, no carcinoma. [REDACTED]

FS3 (C) Prostate urethra, biopsy: No dysplasia, no carcinoma. ([REDACTED])

Clinical History

The patient is an [REDACTED]-year-old man with bladder cancer.

Gross Description

The specimen is received in nine parts, each labeled with the patient's name and medical record number. Parts A-C are received fresh. Parts D-I are received in formalin.

Part A, labeled "right ureter," consists of a single, soft, unoriented ring of red tissue (0.6 x 0.4 x 0.2 cm). The entire specimen is frozen for frozen section diagnosis 1, and subsequently submitted in cassette A1.

Part B, labeled "left ureter," consists of a single, soft, unoriented right of red-pink tissue (0.4 x 0.4 x 0.2 cm). The entire specimen is frozen for frozen section diagnosis 2, and subsequently submitted in cassette B1.

Part C is additionally labeled "bladder and prostate." The specimen consists of a cystoprostatectomy weighing 498 gm and measuring 21 cm right to left, 11 cm anterior to posterior, and 4 cm superior to inferior. The right superior, anterior and lateral wall of the bladder contain a 3.5 x 2.4 x 2.4 cm, firm, white, irregular mass. On the surface the mass is ulcerated. The mass invades 2.4 cm deep through the muscularis propria and touches the perivesicular fat. The mass is grossly 0.2 cm from the perivesicular margin. The mass is 8 cm from the trigone. The remaining bladder mucosa is tan-white and unremarkable. The attached prostate measures 3.5 x 3 x 2.5 cm and contains unremarkable white-tan, whorled parenchyma with no distinct masses. The attached seminal vesicles measure 3 x 2.5 x 1.0 cm each and are both grossly unremarkable. The perivesicular fat is thoroughly searched for lymph nodes, and none are found.

ORIENTED BY: Anatomic landmarks.

INTRAOPERATIVE EVALUATION: Frozen section, urethral margin FS3, cassette C1.

INKING:

- Anterior right surface: Green.
- Anterior left surface: Blue.
- Posterior: Black.

GROSS PHOTOGRAPHS: Yes.

CASSETTES: Cassettes are submitted as follows:

- C1: FS3 urethral margin frozen section remnant.
- C2: Ureter margins.
- C3: Distal prostatic urethra.
- C4-C5: Bladder neck and trigone.

C6-C8: Representative sections showing deepest invasion by mass in right superior-anterior and right lateral walls.

- C9: Representative section of mass transitioning to normal.
- C10: Representative section of bladder dome.
- C11: Representative section of right inferior bladder wall.
- C12: Representative section of posterior-inferior bladder wall.
- C13: Representative section of left anterior-superior bladder wall.
- C14: Representative section of left anterior-inferior bladder wall.
- C15: Prostate, right apex.
- C16: Prostate, right mid.
- C17: Prostate, right base.
- C18: Prostate, right seminal vesicle.
- C19: Prostate, left apex.
- C20: Prostate, left mid.
- C21: Prostate, left base.
- C22: Prostate, left seminal vesicle.

Part D, additionally labeled "left pelvic lymph nodes," consists of multiple pieces of irregular, unoriented, yellow-tan fibrofatty tissue (8 x 7 x 1.5 cm in aggregate) that contain fourteen uniformly tan-yellow soft lymph node candidates (2.8 cm-0.7 cm). The cut surfaces of the lymph node candidates appear homogeneous pink-tan and smooth. All lymph node candidates and representative sections of the fibrofatty tissue are submitted as follows:

- Cassette D1: Largest lymph node candidate, bisected.
- Cassette D2: Second-largest lymph node candidate, bisected.
- Cassette D3: Third- and fourth-largest lymph node candidates, inked and bisected.
- Cassettes D4-D6: Ten intact smaller lymph node candidates.
- Cassette D7: Fibrofatty tissue.

Part E, additionally labeled "right external iliac lymph nodes," consists of a piece of irregular, unoriented, yellow-tan fibrofatty tissue (6.5 x 5 x 1.2 cm) that contains four, uniformly tan-yellow, soft lymph node candidates (1.3 cm-0.6 cm). The cut surface of the largest lymph node candidate appears homogeneously pink-tan and smooth. All lymph node candidates and representative sections of the fibrofatty tissue are submitted as follows:

- Cassette E1: Largest lymph node candidate, bisected.
- Cassette E2: Three intact smaller lymph node candidates.
- Cassette E3: Fibrofatty tissue.

Part F, additionally labeled "right obturator lymph nodes," consists of multiple pieces of irregular, unoriented, yellow-tan fibrofatty tissue (4.5 x 3 x 1 cm in aggregate) that contain four uniformly yellow-tan lymph node candidates (2.5 cm-0.6 cm). The cut surface of the largest lymph node candidate appears homogenous light tan and smooth. The entire specimen is submitted as follows:

- Cassettes F1-F2: Largest lymph node candidate, bisected.
- Cassette F3: Three intact smaller lymph node candidates.
- Cassette F4: Fibrofatty tissue.

Part G, additionally labeled "right internal iliac lymph nodes," consists of multiple pieces of irregular unoriented yellow-tan fibrofatty tissue (2.5 x 2 x 0.8 cm in aggregate) that contain one yellow-tan lymph node candidate (1.4 cm in greatest dimension). The entire specimen is submitted as follows:

- Cassette G1: Lymph node candidate, bisected.
- Cassette G2: Fibrofatty tissue.

Part H, additionally labeled "final left ureter," consists of an unoriented, white-tan tubular segment (1 cm length x 0.7 cm diameter x 0.1 cm wall thickness) with a suture and a metal clip at one end. The serosal surface is smooth and unremarkable. The lumen is patent, without contents. The mucosa is smooth, without discrete lesion. The end with the suture is inked blue. The specimen is serially sectioned and entirely submitted in cassette H1, after removal of the metal clip.

Part I, additionally labeled "final right ureter," consists of an unoriented, white-tan tubular segment (0.9

[REDACTED]

cm length x 0.7 cm diameter x 0.1 cm wall thickness) with a suture and a metal clip at one end. The serosal surface is smooth, without lesion. The mucosa is smooth and unremarkable. The lumen is patent, without contents. The end with the suture is inked black. The specimen is trisected and entirely submitted in cassette I1, after removal of the metal clip.

Diagnosis based on gross and microscopic examinations. Final diagnosis made by attending pathologist following review of all pathology slides.

[REDACTED] pathologist
Electronically signed out on [REDACTED]

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Noted		✓
Case is (circle): <u>QUALIFIED</u> / DISQUALIFIED		
Reviewer Initials <u>SPC</u> Date Reviewed: <u>12/11/13</u>		